

Hysterectomy

A hysterectomy is an operation to remove the womb. It can be used as a treatment for womb, cervical, ovarian, fallopian tube or primary peritoneal cancer (PPC).

On this page

- [What is a hysterectomy?](#)
- [When is a hysterectomy needed?](#)
- [How a hysterectomy is done](#)
- [Before a hysterectomy](#)
- [Recovery after a hysterectomy](#)
- [About our information](#)
- [How we can help](#)

What is a hysterectomy?

A hysterectomy is an operation to remove the womb. The womb (uterus) is a pear-shaped organ where a baby is carried during pregnancy. It is low in the pelvis (the area between the hips).

Image: The female reproductive system

There are two types of hysterectomy:

- **a simple or total hysterectomy** – the surgeon removes the womb and cervix
- **a radical hysterectomy** – the surgeon removes the upper part of the vagina, the cervix, the womb, the pelvic lymph nodes and the tissue surrounding the womb and cervix (parametrium).

Sometimes the surgeon removes other areas at the same time as a hysterectomy. This means the hysterectomy may be one part of a larger operation. For example, you may have an operation to remove the womb, ovaries and fallopian tubes. This is called a hysterectomy and bilateral salpingo-oophorectomy.

When is a hysterectomy needed?

A hysterectomy may be part of surgery to remove:

- [womb cancer](#)
- cancer that started in nearby areas such as the [cervix](#), [ovaries](#), [fallopian tubes](#), [peritoneum](#), or [bowel](#).

If you have [Lynch syndrome](#), a hysterectomy is sometimes used to prevent womb cancer developing.

How a hysterectomy is done

A hysterectomy can be done in different ways. Your surgeon will talk to you about the most suitable type of surgery for you. You may have:

Laparoscopic surgery

The surgeon operates through small cuts in the tummy (abdomen). They use small surgical instruments and a thin telescope with a video camera on the end (laparoscope). The laparoscope lets the surgeon see inside the body.

Abdominal surgery

The surgeon makes one cut (incision) in the tummy. Afterwards, you have a wound that goes across your tummy just above the hips, or that goes down from the belly button to just above the hips.

Robotic surgery

This is like laparoscopic surgery, but the laparoscope and instruments are attached to robotic arms. The surgeon controls the robotic arms.

Vaginal surgery

This is like laparoscopic surgery, but the laparoscope and instruments are attached to robotic arms. The surgeon controls the robotic arms.

Before a hysterectomy

If you [smoke](#), try to give up or cut down before your operation. This will help reduce your risk of chest problems and help your wound to heal after the operation. Your GP can give you advice and support.

You will go to a pre-assessment clinic a few days or weeks before the operation. You will have tests to check you are fit for surgery, such as [blood tests](#) and an [electrocardiogram \(ECG\)](#) to check your heart.

A member of the surgical team will explain the operation to you. Make sure you discuss any questions or concerns you have about the operation with them.

You usually go into hospital on the morning of your operation. You will be given elastic stockings (TED stockings) to wear during and after the operation. These help prevent blood clots in your legs.

Some hospitals follow a prehabilitation or an enhanced recovery programme. This aims to:

- reduce the time you spend in hospital
- speed up your recovery
- involve you more in your own care.

For example, your doctor or nurse may give you information about [diet and exercise](#) before surgery. Or they may put in place any arrangements needed for you to go home. Your doctor or nurse will tell you if an enhanced recovery programme is suitable for you, and if it is available.

Recovery after a hysterectomy

Recovery from a hysterectomy depends on the type of operation you have. Your nurse will give you advice on looking after yourself so that your wound heals and you recover well.

You will also have daily injections of a blood thinning drug to reduce the risk of blood clots. If you have had lymph nodes removed, you will be encouraged to put your feet up when you are sitting. This helps to reduce leg swelling.

Your doctor or nurse can also give you advice about the following:

- **Physical activity and exercise** – try to balance some gentle activity with rest after your operation. Your team will advise you to avoid strenuous physical activity for several weeks to let the wounds heal.
- **Driving** – ask your nurse or doctor for advice about when it will be safe for you to drive. Some insurance companies have guidelines about this. It is best to contact your insurer to check you are **covered to drive**.
- **Sexual well-being** – your wounds may take several weeks to heal. Your surgeon or nurse can give advice about how long to wait if you want to have penetrative sex, or use fingers or sex toys inside the vagina.

If you are having any problems after your operation, it is important to contact your doctor or specialist nurse. We have more information about recovering after surgery for:

- [cervical cancer](#)
- [ovarian, fallopian tube or primary peritoneal cancer](#)
- [womb cancer](#).

You can use our [cancer A-Z](#) to find information about other cancer types.

About our information

References



If you would like more information about the sources we use in our gynaecological cancer information, please contact us at cancerinformationteam@macmillan.org.uk

Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Senior Medical Editor, [Professor Nick Reed](#), Consultant Clinical Oncologist.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

Date reviewed

Reviewed: 01 April 2021 | Next review: 01 April 2024

⚠ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

[Get help](#)